



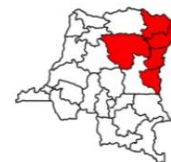
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°114/MVEBDB/06/09/2026

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HIGHLIGHTS (last 24 hours)

As of September 5, 2026, 82 new confirmed cases of Ebola Virus Disease (EVD), including 32 community deaths, were recorded in the provinces of Ituri (49 cases), North Kivu (31 cases), and Haut-Uélé (2). However, 9 deaths intra-CTE were recorded. No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 6,604	ACCUMULATIVE DEATH CONFIRMED 3,175	LETHALITY 48.1%	PATIENTS IN ISOLATION/CTE 851	ACCUMULATIVE HEALED 1,548	TRACKING RATES CONTACTS 85.7% (Of the Day)
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé



61 out of 151
health zones affected
(40.4%)



Summary of key points from the last 24 hours

As of September 5, 2026, **there were 82 new confirmed cases** of Ebola Virus Disease (EVD), including **32 community deaths**. These cases are distributed in the provinces of **Ituri (49 cases), North Kivu (31) and Haut-Uélé (2)**. However, **9 Intra-CTE deaths** were also recorded, including 5 in Ituri and 4 in North Kivu. **bringing the number of deaths for the day to 41.**

In the past 24 hours, **32 patients** have been declared cured after obtaining two negative results for Ebola virus disease (21 in Ituri, 8 in Haut Uele, 2 in North Kivu and 1 in Tshopo).

From the start of the epidemic until September 5, 2026, **there were 6,604 confirmed cases, including 3,175 deaths**, representing a case fatality rate of **48.1 %**. The epicenter remains the Ituri province, accounting for **80.6%** of the country's confirmed cases and **59.8%** of the new cases.

No new health zones have been affected in the last 24 hours.

hours. Thus, **61 out of 151 health zones (40.3%)** remain affected in the 6 affected provinces. Ituri with 28 out of 36 health zones affected, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 5, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	49	5326	2,398	45.0%	28/36 (77.8%)
North Kivu	31	1000	659	65.9%	16/34 (47.0%)
Haut-Uélé	2	249	105	42.2%	6/13 (46.1%)
Tshopo	0	22	9	40.9%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	0	4	3	75.0%	3/11 (27.3%)
Total	82	6,604	3,175	48.1%	61/151 (40.4%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 5, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5326	2398	45.0%	49	15	5	20
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	171	36	21.1%				0
Boga	2	2	100.0%				0
Bunia	1483	462	31.2%	12	6		6
Damascus	30	9	30.0%				0
Drodoro	15	7	46.7%				0
Fataki	77	30	39.0%				0
Gethy	6	2	33.3%				0
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%	1	0		0
Komanda	107	70	65.4%	3	0		0
Lita	237	121	51.1%	3	0		0
Logo	13	5	38.5%				0
Lolwa	25	7	28.0%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	38	21	55.3%				0
Mangala	289	140	48.4%	2	0		0
Mongbwalu	632	290	45.9%	8	2		2
Nia-Nia	226	115	50.9%	5	2		2
Nizi	700	272	38.9%	12	5		5
Nyankunde	124	35	28.2%				0
Rimba	11	4	36.4%				0

Rwampara	985	345	35.0%	3	0		0
Tchomia	60	25	41.7%				0
To ventilate	N / A	366	N / A	N / A	N / A	5	5
North Kivu	1000	659	65.9%	31	17	4	21
Blessed	163	121	74.2%	6	4	2	6
Good	7	2	28.6%	5	1		1
Butembo	204	142	69.6%	7	4		4
Goma	1	0	0.0%				0
Kalunguta	17	8	47.1%				0
Katwa	445	291	65.4%	5	3	2	5
Kayna	1	0	0.0%				
Kyondo	21	15	71.4%	4	3		3
Lubero	6	4	66.7%	1	0		0
Mabalako	8	6	75.0%	1	1		1
Manguredjipa	3	2	66.7%				0
Masereka	15	6	40.0%	2	1		1
Musienene	92	50	54.3%				0
Mutwanga	2	2	100.0%				0
Oicha	8	6	75.0%				0
Vuhovi	7	4	57.1%				0
Haut-Uélé	249	105	42.2%	2	0	0	0
Boma							
Mangbetu	29	14	48.3%				0
Gombari	3	1	33.3%				0
Isiro	92	36	39.1%	2	0		0
Pawa	40	22	55.0%				0
Rungu	2	1	50.0%				0
Wamba	83	31	37.3%				0
Tshopo	22	9	40.9%	0	0	0	0
Bafwasende	2	0	0.0%				0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	3	2	66.7%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	4	3	75.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	2	2	100.0%				0
Total	6604	3175	48.1%	82	32	9	41

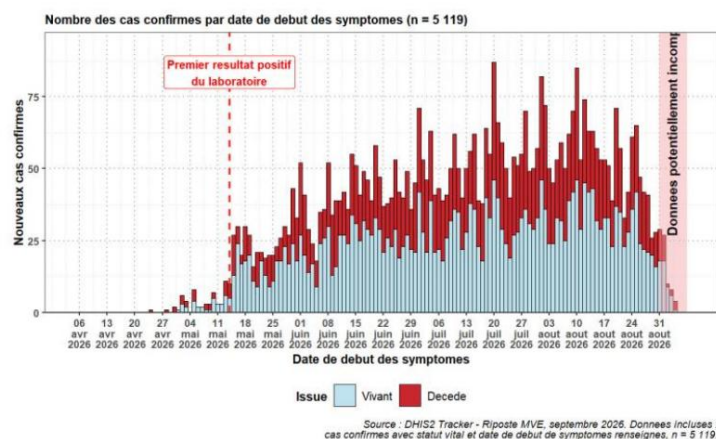
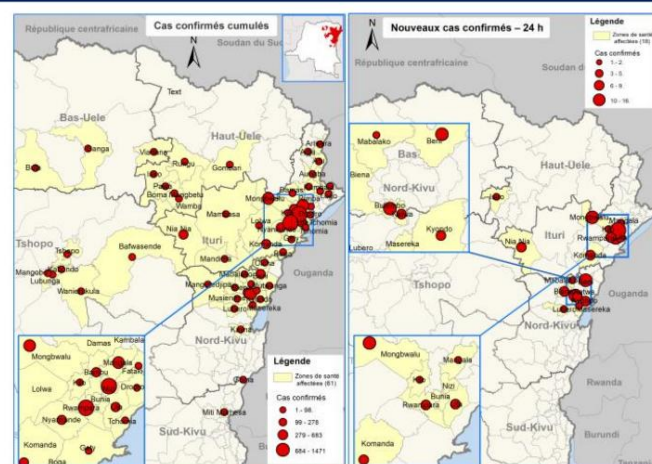
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

** These deaths were recorded in the various CTEs of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of September 5, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	21	2	23	16	2	5	0	18	13
Bas Uélé	27	0	27	3	0	24	0	3	0
Ituri	657	58	715	102	58	355	0	160	83
North Kivu	743	33	776	68	32	629	1	100	38
South Kivu	ND	ND	N / A	ND	ND	ND	ND	ND	ND
Tshopo	86	1	87	4	1	68	1	5	2
Total General	1534	94	1,628	193	93	1081	2	286	136

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

Over the past 24 hours, a total of 1,628 alerts have been recorded in the 5 of the affected provinces (South Kivu did not report), 1,369 (84.1%) were verified. All 286 alerts validated as suspected cases were investigated (100%) and 136 of them were transferred to CT/CTE (Table 3).

The daily proportion, at 85.7% (20,459 views out of 23,880 to follow), remained stable. compared to the previous day (85.9%), slightly above the 85% threshold; 90.9% in Ituri (10,302/11,339), 83.5% in North Kivu (8,491/10,163), 70.4% in Bas Uélé (107/152), 70.1% in Haut Uélé (1,321/1,884) and 69.6% in Tshopo (238/342).

1.1.3. Challenges

Insufficient teams dedicated to surveillance activities and more specifically to contact tracing in the Bas Uélé province, although an improvement has been noted.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

Nine (9) alerts were notified to the various PoE/PoCs as of September 5, 2026. In Ituri, one live alert was confirmed at the Pluto PoC (Mongbwalu Health Zone and District Health Zone), isolated, and then transferred to the Mongbwalu Emergency Treatment Center (ETC). In North Kivu, three alerts were recorded: two bodies intercepted at the Mukulia PoC (Beni District Health Zone), sent to the morgue for swab collection, and one live alert confirmed at the Foner Kasindi PoC (Mutwanga District Health Zone), referred to the Kasindi Health Center for care.

In Bas-Uélé, five alerts were notified, including two at PK65 Sukisa (ZS of Koteli) and three at the PoC of Buta airport; all were invalidated after verification.

Table 4. Status of key indicators for PoC/PoE by province, as of September 5, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé	Overall
Report completeness	87%	100%	55.5%	94%	55.0%	ND	84.6%
% of functional strategic PoE/PoC	9.1%	N / A	N / A	N / A	55.0%	N / A	31.8%
Number of people who have switched to PoE/PoC	128,777	89,946	8,321	15,695	10,869	4,996	258,604
% of travelers screened: 98.0%	98.3%	98.3%	99.0%	97.9%	99.0%	98.2%	
% of travelers who washed their hands	98.0%	98.3%	98.3%	99.0%	98.1%	ND	98.0%
% of travelers made aware	99.0%	98.3%	100%	100%	98.1%	ND	99.0%
Number of alerts notified	1	3	0	0	0	5	9
Number of live alerts validated (suspected cases)	1	1	0	0	0	0	2
Number of bodies recovered today	0	2	0	0	0	0	2
% of suspected cases transferred to CT/CTE	100%	100%	N / A	N / A	N / A	N / A	100%
% of lifeless bodies swabbed/secured	N / A	100%	N / A	N / A	N / A	N / A	100%
% of suspected cases investigated within 24 hours	100%	100%	N / A	N / A	N / A	0%	100%

1.2.2. Challenges

Operationalization of PoE/PoC not optimal: Completeness also remains low in South Kivu (55.5%) and Haut-Uélé (55.0%), where five PoE/PoC (Doko, Dusu, Apodo, Aungba and Gossamu) did not transmit their data due to lack of connection, and Bas-Uélé did not report its completeness indicators; in Tshopo, only 18 of the 30 planned PoCs are activated (60%); persistence of refusals, with 2,581 refusals of screening and 2,998 refusals of hand washing in Ituri and 228 refusals of screening in Haut-Uélé, motivated by rumors of water poisoning and thermal scanners.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 49 new positive results** (34 living and 15 deaths) out of 292 new samples received and analyzed (**positivity: 16.8%**).
- **North Kivu : 31 new positive results** (14 living and 17 dead) out of 96 samples analyzed (**positivity of 32.3%**).
- **Haut-Uélé: 2 new positive results** (2 live) out of 8 samples analyzed (**positivity: 25%**).
- **Bas-Uélé: 3 samples** received and tested (3 live), **all came back negative**.
- **Tshopo: 5 samples** received and tested (3 live), **all came back negative**.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 18 of the 60 planned infection control zones (30.0%) were opened, 50 of the 55 identified households were decontaminated (90.9%), and 1 of the 4 social and medico-social establishments (ESS) were decontaminated (25.0%); 15 infection prevention and control (IPC) kits were distributed to households. Regarding the health and safety database (HSD), 104 alerts were recorded, including 7 postponements from the previous day, 71 HSDs were carried out, and 56 bodies were swabbed, with 30 bodies unable to be swabbed due to community resistance.
- **In North Kivu**, 10 of the 30 expected rings were opened (33.3%), 6 of the 21 identified households were decontaminated (28.6%), and 21 of the 28 health facilities were decontaminated (75%), without the provision of kits to households. The EDS pillar received 43 alerts, carried out 37 EDS and swabbed all 43 bodies (100%), without any failures.
- **In Haut-Uélé**, all 3 planned infection control zones were opened (100%), 16 out of 16 identified households were decontaminated (100%), and all 8 health facilities were decontaminated (100%); 53 service providers were briefed on basic infection prevention and control (IPC) in Isiro (45), Wamba (5), and Pawa (3), and 15 health establishments were decontaminated in the 6 affected health zones. Five emergency response alerts were recorded, 5 IRCs were carried out, and 5 bodies were swabbed.
- **In Bas-Uélé**, a briefing was organized for 44 healthcare providers in 15 health centers on hand hygiene, preparation of chlorinated solutions, waste sorting and management, PCI assessment of 2 health centers (CS Bale, 14%, and CS du Congo, 30.2%) and supervision of PCI/WASH activities at PoC Bale, in the AS of Bale (ZS of Buta).
- **In Tshopo**, monitoring and support of 7 ESS with the briefing of 56 service providers on PCI measures and supply of 500 litres of water at the Madula checkpoint; the only EDS alert, received in the Kabondo health zone, could not be carried out due to community resistance.

1.4.2. Challenges

- **Weak implementation of PCI standards:** in Ituri, the opening of the rings remains incomplete (30.0%, 18/60), decontamination teams remain insufficient in Fataki, Mangala, Nizi and Komanda and Mandima has been deprived of activities due to lack of inputs; in North Kivu, only 6 of the 21 identified households have been decontaminated (28.6%) and reporting is complete for only 7 of the 15 health zones (46%); in Bas-Uélé, the waste area of the Buta General Referral Hospital is still not functional and sorting units are lacking at the Ganga and Viadana General Referral Hospitals.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 100 new admissions to the various dedicated facilities, compared to 85 discharges, including 21 recoveries. Thus, 510 patients are hospitalized in 978 beds, representing an occupancy rate of 52.1%.
- **In North Kivu**, 54 new admissions and 38 discharges were recorded, including 2 recoveries; 253 patients are hospitalized expressing an overall bed occupancy rate in CTE/CT is in oversaturation (115.0%; 220 beds available).
- **In Haut Uélé**, there were 15 new admissions compared to 7 discharges (8 recovered). Thus, 65 patients are hospitalized for 128 beds, representing an occupancy rate of 50.8%.
- **In Tshopo**, one new admission was recorded and two discharges (one recovered). Six (6) Patients are in isolation in 25 beds, representing an occupancy rate of 24.0%.
- **In South Kivu**, seventeen (17) patients are in isolation out of 25 beds, representing an occupancy rate 68.0%

1.5.2. Challenges

Low capacity and saturation: in North Kivu, the overall occupancy rate reaches 115.0% and some patients continue to be treated in non-standard structures; in Ituri, the CTE/CTs of Nizi, Mambasa and Mandima are saturated on the side of suspected cases, 39 patients are in critical condition and the extension work of Nizi and construction of Bambu remain in progress; in Bas-Uélé, the absence of CTE/CTs and ambulances limits the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

Vaccination with the ERVEBO vaccine continued in Bas-Uélé, in the Buta health zone, benefiting 500 frontline workers and members of the Integrated Management System (IMS), bringing the remaining stock to zero. 484 of the 500 vaccinated individuals (96.8%) have been recorded in DHIS2, and 104 minor adverse events following immunization (AEFIs) have been reported to date, with no serious AEFIs. A supplementary supply of Ervebo doses has left Kisangani for Buta via the Isiro-Buta road, for deployment in the six priority health zones.

1.6.2. Challenges

Stock shortage of Ervebo in Buta leading to a temporary suspension of vaccination of PPL and TPL, lack of mobility for vaccination teams in advanced strategy and lack of micro vaccination plans for PPL in the 6 priority health zones of Bas-Uélé (Aketi, Ango, Bili, Buta, Ganga and Viadana).

1.7. Continuity of Care

In Bas-Uélé, post-exposure prophylaxis of high-risk contacts continued in the Ganga health zone and support was given to an awareness session for the CACs of the Baruti health district on the causes and challenges of raising alerts.

In Bas-Uélé, 16 high-risk contacts remain under prophylaxis in the health zones of Buta (10) and Ganga (6), with no new admissions or discharges, and the Buta General Referral Hospital has been assessed (PPE shortage and lack of triage area).

In Haut-Uélé, finalization of the database of the 20 PPLs identified in the 6 affected health zones.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, a meeting was held by the head of the Bayaku group, which comprises 11 villages, at the end of which the village chiefs pledged to support the activities of

Communication; raising awareness of adherence to EDS practices around 30 of the 35 community deaths reported in Nizi (15), Nyankunde, Fataki, Komanda and Mongbwalu; 11 educational talks, 219 home visits, 164 mass awareness sessions and 21 radio broadcasts produced

- **In North Kivu**, 3,325 home visits reached 10,200 people and 67 talks were held educational 1,699 people; a briefing (20 people), 2 advocacy sessions and 14 mass communication actions in schools (5,566 people) were organized; 7 of the 10 feedbacks captured and 2 of the 4 incidents recorded were resolved within 48 hours.
- **In Tshopo**, 1,205 people were sensitized (674 women and 531 men), 47 people trained in CREC in Bafwasende, 211 leaflets distributed, 2 community feedbacks collected and 1 out of 2 refusals addressed, with communication on prevention at Mont Fort College (Makiso-Kisangani Health Zone)

1.8.2. Challenges

Persistence of community resistance: in Ituri, 19 refusals of isolation at the CTE calling for reinforcement of the CREC and 30 bodies not swabé due to community resistance in Nyankunde, Rwampara, Nia-Nia, Tchomia, Mangala, Mandima and Fataki; in North Kivu, threat of fire to the morgue of Ndindi (Beni) after the removal of the body of a confirmed case and contact of the population with a confirmed body at the cemetery of Rwahwa (Katwa); in Haut-Uélé, categorical refusal of an escapee found in Nduka (ZS of Isiro) to return to the CTE.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In North Kivu**, 27 new confirmed cases and 27 of the 100 suspected cases benefited from SMSPS interventions; 71 results were announced, including 27 positive cases; 49 people with primary AIDS (PLA) received support, 42 households were assisted, and 16 recoveries were monitored in the community. In North Kivu, 21 confirmed cases accompanied and 21 children followed in community and 3 recovered reintegrated, In Haut-Uélé, communication actions around the last 5 confirmed cases reached 71 households in Isiro, Pawa and Wamba.

1.9.2. Challenges

In Ituri, low coverage of SMSPS interventions around the latest confirmed cases in the health zones of Bunia, Mangala and Komanda; in North Kivu, insufficient human resources and lack of means of mobility for actors of the pillar at the provincial and zonal levels.

1.10. Logistics

1.10.1. Main actions

- **In Ituri**, two inter-CTE transfers were organized, one to the Nizi CTE and the other to the Bunia General Referral Hospital, resulting in the evacuation of 15 patients and the provision of food aid to 469 of the 485 hospitalized patients (96.7%); two ambulances are out of service, one in Bambu and one in Nizi.
- **In North Kivu**, 12 calls were received, resulting in 12 successful missions (100%), with 7 ambulances mobilized out of 11 available (64%); 139 packages were received from the INSP, and 9 packages of laboratory supplies donated by the WHO were in transit to Goma (4). Bukavu (5), delivery of IPC supplies to Beni and medicines to Musienene, and continuation of the construction sites of the CTE at Matanda Hospital and the CT at Musienene General Referral Hospital
- **In Bas-Uélé**, provide support for the mobility of health zone teams for the verification of alerts and packaging of IPC inputs and care intended for the Ganga health zone.

1.10.2. Security

1.10.3. Main actions

- In North Kivu, seven activities to maintain public discipline were carried out: accompanying EDS teams for the burial of 2 confirmed cases at the Bundji cemetery, securing a lifeless body in the Boiken AS and the burial of 2 confirmed cases at the Masiani family cemetery, as well as covering IPC teams at the Maranatha CS and the Divine Healing and Bethsaida dispensaries (Tamende and Bundji AS).

In Bas-Uélé, monitoring is underway to ensure the availability of personnel and the strengthening of protection measures for agents involved in the response.

1.10.4. Challenges

Nineteen (19) refusals of isolation at the CTE (Treatment and Education Center) were reported in Ituri. In North Kivu, there was a threat of fire at the Ndindi morgue in Beni following the removal of the body of a confirmed case, and the population came into contact with a confirmed body at the Rwahwa cemetery in Katwa. In both cases, security forces were alerted late.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions.

- In Ituri, 5 health workers received PSEA briefing and signed the code of conduct in the health zones of Bunia, Mambasa, Komanda, Nyankunde, and Lolwa; 10 people were sensitized in the community, and 13 of the 36 health zones (36.1%) are now familiar with the reporting mechanism, with 2 partners (WHO and UNICEF) reporting their PSEA activities

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES	TIMELINE OF KEY EVENTS
• Maintaining performance in epidemiological investigations (100% of suspected cases investigated) and a stability in the daily proportion of contact tracing (85.7%) which remains above the threshold of 85.0%. • Continued reporting in the provinces of Ituri, North Kivu and Haut-Uélé highlights sustained community transmission of the disease). • As of September 5, 2026, 82 new confirmed cases, including 32 community deaths and 9 intra-CTE deaths, were recorded in the last 24 hours, bringing the national total to 6,604 confirmed cases and 3,175 deaths (case fatality rate: 48.1%). • Ituri remains the epicenter of the epidemic, accounting for 80.6 % of cumulative confirmed cases and 59.8% of new cases reported in the last 24 hours, while the province of North Kivu holds 37.8% of new cases with a high lethality. • Strengthening interventions (surveillance, care, IPC, logistics and community engagement) remain essential to quickly interrupt transmission chains. Priorities	• From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo. • Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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